جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : فاطمة سعد ابراهيم حبيب

المحافظة : الجيزه **الحي / المدينة :** الهرم **العنوان :** 5ش-على حنفى-الطالبية-الهرم-الجيزة

رقم البطاقة : 27007042300701 **رقم التليفون :** 01100105198 **تاريخ تسجيل الإستمارة :** 18-10-2026

العمر(تاريخ الميلاد) : 04-07-1970 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي المقر بما فيه

التوقيع:(----------) **التوقيع:(----------)**

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج الالم ومضادات القئ لمرضي الاورام المنتشر [14456] **قسم العلاج :** الاورام

التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------

الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------

خطة العلاج : ----------

المستشفي : ----------**قسم :** ----------

جهة العلاج : ----------**نفقات :** ----------

ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فاطمة سعد ابراهيم حبيب
National ID: 27007042300701

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

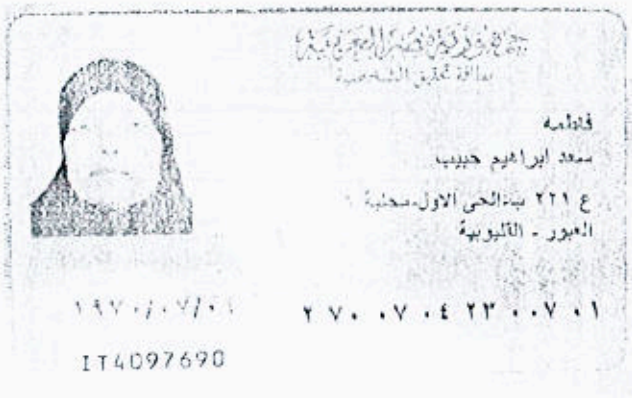
نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Pin Medication And Anti Emetics

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين

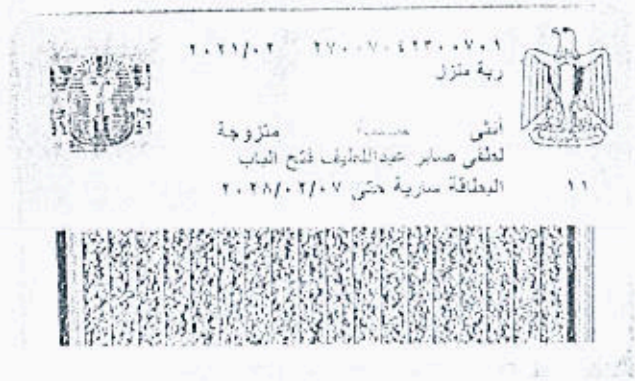


بطاقة تحقيق الشخصية

فاطمه
سعد ابراهيم حبيب
ع ٢٢١ بناالحى الاول-سحلية
العبور - القليوبية

٢٧٠٠٧٠٤٢٣٠٠٧٠١ ١٩٧٠/٠٧/٠١

IT4097690



٢٧٠٠٧٠٤٢٣٠٠٧٠١ ٢٠٢١/٠٢
ربة منزل

انثى متزوجة
لطفى صابر عبدالعليف فتح الباب
البطاقة سارية حتى ٢٠٢٨/٠٢/٠٧

١١

عاجل

DMS

كاربوتاكسول

National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف روسي الترناشونال

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

البنت المصرية هي صحة مصر

MDT FORM 31 /12 /2025 G

Name	خاجة سعد ابراهيم حبيب	**MRN**	3221 / 2018
		ID	٢٧٠٠٧٠٤٣٠٠٧٠١
Age	55 yrs	**Phone**	٠١٠٠٩٩٩٩١٨٣
Menopausal status	Post	**Marital status**	
NRS SCORE		**Family history**	
BMI		**REASON OF MDT**	DP
CO morbidity	Free	PS 2 c/o severe Rt hyp. Chondrial pain	

Clinical Data

(Rt)

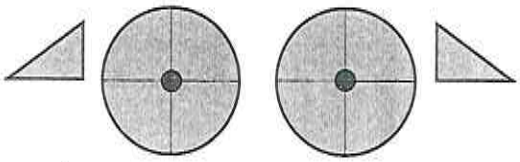
BX: IDC

ER:	PR:	HER 2:	Ki67:
0/8	5/8	Not amplified	

Sonomammography:

Mets Workup:

(R) L



TNM - liver - lung

Recommendations: (2019) Rt MRM (T2N0M0) → AC X4 / T+XL 12 wks RT then TAM From (10/2019) to (7/2022) Then shifted to AI (Femara)

10/2024 Abd pain → CT → HFL, Then started Fulvestrant + Ibrance for 3 Cycles (From 1/2025 to 4/2025)

(26/4/2025) CT → Newly developed small pulmonary nodules So started Xeloda From 8/2025 till now

MDT OUTCOME & PLAN CT CAP (23/11/2025) → progression of size of HFL + Peritoneal nodules - New developed Splenic FCL lesion

(7-12-2025) CA 125 (36,9) . Lt. adnexal mass → TV/US, CA-125

→ Carboplatin / Gemcitabine every 3 weeks (R)

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
		No R 3/12/2025			

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: فاطمة سعد ابراهيم	
MR. : 194	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالتها

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انها

MDT 31/12/2025 for gemzar/carboplatin C1D1 GEMZAR 800MG/M2 CARBO AUC 5

WT 46 / HT 167 pharmacy visit for C1D8 gemzar 1170mg

MBC TO LIVER WT 46 / HT 167 for C2D1 AND D8 PHARM gemzar 1170mg PENDING LAB

Pharmacy visit For Cycle 2 day 8 Gemzar Labs 18/2 ... accepted

for c3 d1 gemzar/ carbo d8 gemzar pharmacy cbc ok pending function

Pharmacy Note Wt:46, Ht:167, BSA: 1.46 For C4 D1 Gemzar 800mg/m2 (1170) + Carboplatin AUC5 Labs 15/4 ... accepted

For C4 D1 Gemzar 800mg/m2 (1170) + Carboplatin AUC5 Labs pending ... accepted

55 yr old female, postmeno, medically free, hx since 2019 of rt breast ca IDC, underwent RT MRM T2 N0 M0, adj AC X4 taxol 12 wks, PORT then adj TAM from 10/2019 till 7/2022, then shfited to femara for 2 yrs till 2024, in 10/2024 she presented with abd pain CTs > HFL >> st faslodex + ibrance for 3 cycles from 1/2025 till 4/2025 ... 26/4/2025 newly developed small pulm nodules so pt started xeloda from 8/2024 till 12/2025, CT CAP 11/2025 PD of HFL + newly dev peritoneal nodules + splenic focal lesion + lt adnexal mass (CA 125 >>> 36.9), MDT 12/2025 > Start carboplatin + gemzar with reg assessment NEW CT CAP 21/4/2026 > WITHOUT COMPARISON (pulm nodules measuring 7 mm, HFL measuring 5x 4.5 cm, no IHBRD, left adnexal mass with calcification likely dermoid 4x4 cm) FOR COMPARISON today for C4D8 gemzar only

prem mbc to liver med ln left adnexa er0/8 pr 5/8 her22+ sish not amplified hfl biopsy 9/2024 .history rt mrm 1/2019 t2 no mo rec 4 ac 12 taxol tam x2 shifted to femara x2 till9/2024 liver mets biopsied rec fasoldex ibrancse from 10/2024 till 5/2025 dp newly dev pul noddules rec xeloda till12/2025 dp new cts 26/4/2026 bilobar hfl largest 5x4.5 (prev 8/2025 11.5x5.7 cm) left adnexal cyst 1x1 (prev 5.6x6.9 8/2025) pul nod 7 mm for ccomp meanwhile for c5 d1 gemzar1400 1 gm /m2 carbop auc 5

cts with comp 21/4 sd of multiple bilobar hfl largesr at seg 7,8,5,4 m 10.9x5.7x10 bilat pul nod 7 mm sd for continuation then redo cts after 1 m . for c5 d8 gemzar 1400

for c6 d1 gemzar C6 carbo auc 5after labs + supp ttt (add PPi , geanytrl , paracetamol) C6d8 gem >> pharmacy CT CAP e comparision >> R

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : **dina makram**

يعتمد ,

for c6 gemzar 1400 carbopl auc 5 and repeat cbc next week hb 8.9 ppi 1x2

for c7d8 gemzar 1600 and 1 proph neupogen cts with comp pending

cts on 6/8/2026 sd of pulmonary nodules largest 7 mm multiple hfl largest in rt lobe 7 ,8,5 ,4 m 10.9 x5.7 x10 cm for radiological referral as same sizes of last cts 4/2026 for c7 d1 gemzar 1400 carbopl auc 5

Patient Name	: الأستاذه / فاطمه سعد ابراهيم	Request No.	: 13002390
Patient No.	: 27007042300701	Request Date	: 28/10/2024
Path No.	: 4136/24	Verification Date	: 19/11/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 54 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Known case of breast cancer with hepatic focal lesion.
Nature of Specimen: Tru-cut biopsy, collected at 27/10/2024.

Gross Examination:
- Four grayish white tissue cores up to 1 cm, totally submitted.

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, Her2, CK7, GATA 3, SOX 10 & PAX 8.

Microscopic Examination:
* Histological type: Metastatic carcinoma, Positive for CK7 and focally positive for PR, Suggestive of breast origin.

* IPT (Positive control showed appropriate reactivity):
- GATA 3: Negative.
- PAX 8: Negative.
- ER: No nuclear reaction in the tumor cells (score 0/8).
- PR: Weak nuclear reaction in 60% of tumor cells (score 5/8).
- Her2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
-CK7: Positive.
-SOX10: Negative.

Diagnosis:
Known case of breast cancer with hepatic focal lesion, Tru-cut biopsy:
Metastatic carcinoma, Positive for CK7 and focally positive for PR, Suggestive of breast origin.
IPT:
- GATA 3: Negative.

Verified by :

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / فاطمه سعد ابراهيم	Request No.	: 13002390
Patient No.	: 27007042300701	Request Date	: 28/10/2024
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Age	: 54 Y	Department	: Out Patient
Referreing Doc :		Registered by	: marwa mohamed youss

Pathology Report

- PAX 8: Negative.
- ER: Negative (score 0/8).
- PR: Positive (score 5/8).
- Her2: Equivocal for SISH.
-CK7: Positive.
-SOX10: Negative.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10%of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature
Dr. Salma Tarek.
Dr. Toka Medhat.

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY INTERNATIONAL EGYPTE

Patient Name: Fatma Saad Ibrahim Habib	Date : 07/12/2025
Exam : ultrasound TRANS VAGINAL	Age : 55 Year(s)
Medical ID: 3221/2018	Pat. ID: 41070

Trans-vaginal ultrasound scanning revealed: -

Uterus: -

- *AVF, bulky uterus with lobulated outline showing multiple intramural upper and lower corporeal fibroids causing contour bulge, the largest one is seen measuring 4x3.5cm showing calcific focus*
- *The endometrial line is 5mm.*
- ***No gross adnexal lesions could be detected for better assessment MRI is recommended***

No free fluid in the Douglas pouch.

Conclusion:
Uterine fibroids

Signature Dr . Nermien Ramadan
07/12/2025 4:13 PM

Ultrasound
[[DOCTYPE:ULTRASOUND]]

- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries
- **stationary** Subhepatic peritoneal nodule 14 x11 mm.
- Enlarged spleen with homogenous parenchymal texture **showing upper lobe focal lesions 9 x 7 mm .** Small spenule is also noted 11 x 9 mm.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- **No appreciable changes** regrading the left adenxal likely ovarain hetrohenously enhancing mass lesion seen measuring about 5.6x6.9x5.3 cm in maximum AP, TS and CC diemsnions, it shows internal calcific foci for MRI/US correlation.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic collections.

IV Scanned skeleton:

No definite worrisome lytic or sclerotic focal lesions.

Dorsolumbar spondylosis with lumbar scoliosis convex to left side.

Mild bilateral sacro-iliitis.

Opinion:

Reported case of right breast cancer showing;

- **Newly developed splenic focal lesion**
- **Course of the size of hepatic mass .**
- **Rather stationary rest of the study.**

DR/Reham Khedr,,,MD

Signature reham kheder
26/11/2025 9:56 AM

CT Scan
[[DOCTYPE:CT_SCAN]]

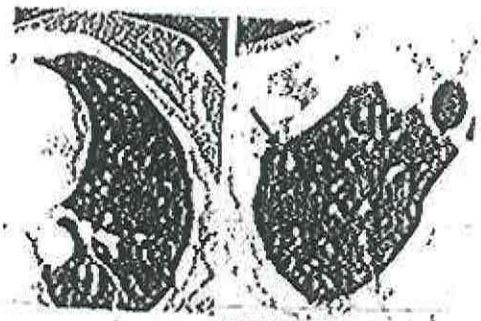
alfa
SCAN

Name: Fatma Sied Ibrahim ID: 4206566 Date: 26/04/2025

Multislice **post contrast CT scan of the chest, abdomen and pelvis with multiplanar reformatting revealed:**

- **Status post-operative and therapeutic for a case of right breast cancer with pathologically proven hepatic metastases.**
- **Reviewing the previous CT study dated 11/12/2024.**

I-CHEST:



- **Newly developed** few small pulmonary nodules seen at lingula, right upper and middle lobe (the largest seen at right upper lobe measures 3.0 mm). **Still noted** few scattered bilateral pulmonary nodules, largest is seen at middle lung lobe measures about 3 mm.
- No pulmonary consolidation, cavitation or bronchiectatic changes.
- **Still noted**
  - Prominent paratracheal, para-aortic, subcarinal and right hilar lymph nodes are noted, the largest measuring about 1.2 x 2.2 cm at subcarinal group.
  - Prominent left internal mammary lymph nodes, the largest measuring about 8 x 5 mm.
  - Right anterior subpleural pulmonary reticulations, likely post radiotherapy changes.
  - Mosaic attenuation of the lung bases suggestive of small airway disease.
  - Middle lobe, lingula and bilateral lower lobes atelectatic bands.
  - Few small subcentimetric right supraclavicular lymph nodes.
- No pleural or pericardial collections seen.
- No gross cardiac enlargement.
- No evidence of mastectomy operative bed gross masses or collections.

CT Scan
[[DOCTYPE:CT_SCAN]]

alfa
SCAN

II-ABDOMEN & PELVIS:



- **No gross interval changes** regrading the multiple variable sized discrete and confluent hypodense heterogeneously enhancing hepatic focal lesions, the largest is seen involving segments (IV, VIII, V & VI), measuring roughly about 6 x 10.8 cm with overlying capsular retraction.
- **Still noted**
  - Mild peritoneal stranding is seen beneath anterior abdominal wall anterior to right hepatic lobe.
  - Moderately enlarged spleen. No sizable focal lesions.
  - Gall bladder body/neck tiny mural based densities. For US correlation.
  - Enlarged porta-hepatis, porto-caval, para-aortic, aorto-caval and mesenteric lymph nodes, the largest one seen at porto-caval group measures about 1.6 x 3 cm.
- No biliary radicles dilatation.
- Normal CT appearance of the pancreas, suprarenal glands, kidneys, aorta, and IVC.
- Partial filling of the urinary bladder showing minimal circumferential mural thickening.
- Bulky uterus showing inhomogeneous enhancement & lobulated outline in the virtue of few fibroids with related calcific focus, for US correlation.
- No ascites.

OPINION:

- **No gross interval changes regrading the multiple heterogeneously enhancing metastatic hepatic focal lesions.**
- **New developed few small pulmonary nodules, as described.**
- **No appreciable changes as regard the rest of the study.**

Dr. Ahmed Zein, MD

Dr. Omnia Abd Al Fattah, MD

ID: 4204353

CT Scan
[[DOCTYPE:CT_SCAN]]

د . عصام علي الشيخ
دكتوراه الأشعة التشخيصية
أستاذ بكلية طب القصر العيني

مركز الطاهرة للأشعة
TAHRA RADIOLOGY CENTER
EST 1999

د . هاني أحمد شاهين
دكتوراه الأشعة التشخيصية
أستاذ بكلية طب القصر العيني

Name: Mrs. Fatma Saad Ibrahim Habib

Date: 3/11/2024 ID:2224068214 Acc.:5044181

Referring / Hospital Harm

Tc-99m MDP whole body bone scan

Technique:

- Whole body images were acquired 180 minutes after IV injection of ~15 mCi of the tracer.

Findings:

- Uniform tracer uptake all over the surveyed skeleton is noted with no evidence of any active spots.
- Degenerative changes of the spine are seen.
- Arthritic knee changes are noted.
- Both kidneys are visualized.

Opinion:

- *No scintigraphic evidence of metastatic osseous deposits.*

Truly yours
Prof. Dr. Ahmed Kandeel. MD
Prof. of Nuclear Medicine, faculty of medicine, Cairo University

HOTLINE 19173
G-CERT

Patient's Name: **Fatima S Habib**
Date: **2026-08-06**
Referred by: Prof.Dr. **no doctor**
Hospital: **Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST INJECTION REVEALED: AS COMPARED TO PREVIOUS STUDY DATED21/4/2026, THE CURRENT STUDY SHOWED

I- CHEST:

- Still seen multiple bilateral pulmonary nodules the largest is seen in the apical segment of the left lower lobe measuring 7 mm
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No dilated biliary radicles. Multiple hepatic hypodense focal lesions the largest is seen in the right lobe segment VII, VIII, V& IV measuring 10.9X5.7X10cm showed heterogenous mainly peripheral enhancement
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Enlarged spleen
- Normal C.T appearance of the pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- **overall stationary course.**

Dr.Asser Ahmed

Patient's Name:	**Fatima ؟ Habib**
Date:	**2026-08-06**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS AFTER ORAL AND I.V. CONTRAST INJECTION REVEALED: AS COMPARED TO PREVIOUS STUDY DATED21/4/2026, THE CURRENT STUDY SHOWED

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- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- **overall stationary course.**

Dr.Asser Ahmed